CKD Monitoring & Care Pathway

Synthetic reference for demo purposes — general guidance, not a clinical guideline or St. Luke's policy.

Monitoring frequency (eGFR and urine ACR)

- **G1–G2 (with damage markers):** at least annually.
- **G3a–G3b:** every 6–12 months (more often if progressing).
- **G4:** every 3–6 months.
- **G5:** every 1–3 months, with nephrology co-management.

Targets & management

- **Blood pressure:** aim < 130/80 mmHg where tolerated.
- **Glycemic control** in diabetes per individualized targets.
- **Avoid nephrotoxins:** NSAIDs, contrast when possible; adjust renally-cleared drug doses.
- **Address complications:** anemia, mineral-bone disease, acidosis as stages advance.

Documentation

Ensure the CKD stage is recorded in the problem list. Undocumented or mis-staged CKD is a common gap: reconcile the lab-derived stage with the documented stage and update the record so the whole care team sees a consistent picture.